MINDSET HEALTHCARE

Go-to-Market & Operations Strategy

> *This section supplements the business plan and pitch deck. Figures marked as illustrative are placeholders pending validation against real pilot data, and should be reviewed alongside the financial plan before distribution to investors.*

1. Organizational Roles & Management Structure

Mindset Healthcare's technology and operations function is organized around two core mandates: designing and building the technology platform, and running day-to-day service delivery through Marketing and Referral roles.

1.1 Core Functional Roles

- Marketing — brand building, campaign execution, and channel partnerships.
- Referral — managing the pipeline of patient referrals into partner hospitals and providers.

1.2 Management Functions

- Planning — roadmap and resourcing across the platform and provider network.
- Budget — cost control and spend allocation across functions.
- Forecasting — demand and revenue projections used to guide hiring and marketing spend.
- Referrers — relationship management with the clinicians and partners who refer patients.
- Clinician — clinical oversight and quality assurance across the provider network.

1.3 Departmental Structure

Beneath management sit two departments — Finance and Marketing — each with its own reporting lines:

- Finance — Accounts and POS (point-of-sale) machines, covering payment processing and reconciliation.
- Marketing — Branches, split into internal and external positioning models, covering how the brand is presented to internal stakeholders (providers, staff) versus the external market (patients, diaspora, partners).

> *The founder/team org chart in the source notes references named individuals and an acceptance date (18th) that should be confirmed and inserted here before this section is finalized.*

2. Client Acquisition Build-Out

The client-facing (patient-side) go-to-market motion centers on a lightweight digital funnel supported by a time-boxed paid campaign, channel-specific social media strategies, and organic outreach to clinicians.

2.1 Digital Funnel

- Website with a clear value proposition and calls to action.
- Landing page with an integrated lead capture form.
- Social media presence to drive awareness and traffic into the funnel.

2.2 Launch Campaign

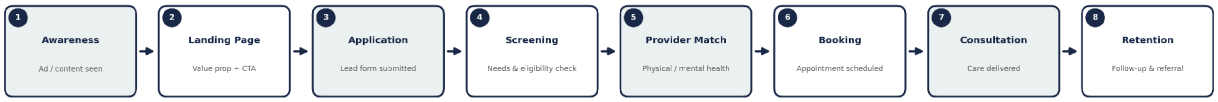
An initial paid social media marketing campaign is planned to run over a 4-week period.

- Duration: 4 weeks.
- Budget: ₦5,000 weekly for the 4-week period (illustrative — to be validated against actual channel costs and CAC targets).

> *The budget figure above is a placeholder carried over from planning notes and should be reconciled with the bottom-up financial assumptions in the business plan before this document is finalized.*

2.3 Patient Application Funnel — Detailed Structure

Every channel feeds into the same underlying funnel, taking a prospective patient from first exposure through to a booked consultation and long-term retention. Eight stages give the team clear ownership and measurable drop-off points at each handoff.



- Awareness — prospect sees a paid ad, organic post, or referral mention.
- Landing Page — clicks through to a page stating the value proposition (dual physical + mental health access) with a single clear call to action.
- Application — completes a short lead form: name, contact, location, and stated need (physical, mental health, or both).
- Screening — care coordinator reviews the submission for urgency and eligibility, and reaches out within a set response window.
- Provider Match — patient is matched to the right provider in the network, physical or mental health, based on need and location.
- Booking — appointment is scheduled directly with the matched provider or hospital.
- Consultation — care is delivered, either in person or via the platform's referral pathway into a partner hospital.
- Retention — post-visit follow-up, satisfaction check, and prompts for repeat use or referral to family/friends.

> *Each stage should carry a target conversion rate once pilot data is available, so drop-off can be diagnosed stage-by-stage rather than only at the top and bottom of the funnel.*

2.4 Channel Campaign Portfolios — Dual-Role Strategy

Every channel in the plan is asked to do two jobs at once: acquire patients directly, and recruit the clinicians and hospitals who refer patients in. Facebook, WhatsApp, Telegram, and LinkedIn each carry two campaign portfolios below — Portfolio A for patient acquisition, Portfolio B for clinician/referrer outreach — with its own audience,

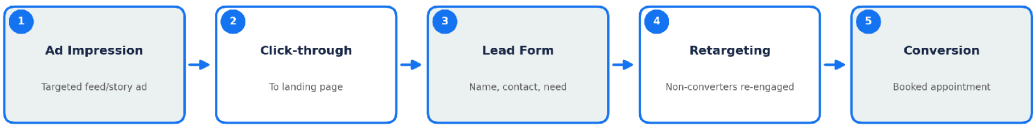
content plan, funnel, and KPIs. Role-B portfolios share a red funnel treatment throughout this section so the referrer-recruitment thread is easy to trace across every channel.

2.4.1 Facebook & Instagram

Facebook runs as the primary paid-media engine for patients and doubles as a low-cost route into clinician-facing medical community groups.

Portfolio A — Patient Acquisition

Objective	Drive qualified leads into the application funnel via paid + organic social.
Audience	Urban Nigerian adults 24–45; lookalike audiences layered in once initial conversions are logged.
Content Pillars	Short video and carousel ads contrasting Mindset's dual physical + mental health offering against the fragmented status quo; patient testimonials once available; educational posts on when to seek physical vs. mental health care.
Cadence	3–4 organic posts/week; always-on ad set, creative refreshed every 2 weeks to avoid fatigue.
Budget (illustrative)	~~₦~~3,000/week of the ~~₦~~5,000/week test budget — primary paid channel.
Sample CTA	"Physical or mental health — one number gets you the right doctor. Book your first visit today."
KPI Targets	Cost per lead; landing page conversion rate; cost per booked appointment.



Portfolio B — Clinician & Referrer Outreach

Objective	Recruit doctors and hospital administrators into the referral network via Facebook's medical professional groups.
Audience	Nigerian doctors, hospital admins, and clinic owners active in professional Facebook groups.
Content Pillars	Case studies on referral volume and patient outcomes; explainer posts on how the provider network and payout model work; short founder-led videos on the mission.
Cadence	1–2 posts/week into relevant groups; sponsored posts optional once initial traction is proven.
Budget (illustrative)	Organic by default; optional ~~₦~~500/week to boost posts into medical groups.
Sample CTA	"Send patients you can't see today to a network you can trust. Partner with Mindset Healthcare."
KPI Targets	Group engagement rate; DMs initiated by clinicians; discovery calls booked.

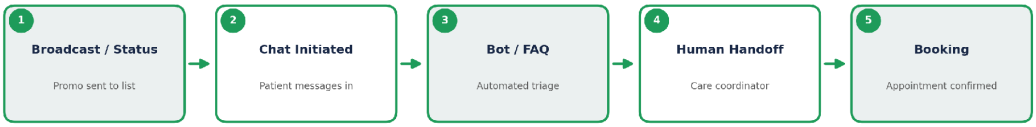


2.4.2 WhatsApp

WhatsApp is the highest-trust channel for a Nigerian audience on both sides of the network — patients booking care and doctors managing referrals.

Portfolio A — Patient Acquisition + Support

Objective	Convert warm interest into booked appointments and handle ongoing care coordination.
Audience	Prospective and existing patients, segmented by interest (physical vs. mental health).
Content Pillars	WhatsApp Business catalog of services; status updates with health tips; broadcast lists segmented by need.
Cadence	2–3 broadcasts/week; community groups moderated daily by the care team.
Budget (illustrative)	Organic — no paid spend; cost is care-coordinator time.
Sample CTA	"Type 'BOOK' to speak with a care coordinator now."
KPI Targets	Chat-to-booking conversion rate; first-response time; repeat-contact rate.



Portfolio B — Clinician & Referrer Outreach

Objective	Onboard doctors into an active referral workflow through direct relationship-building.
Audience	Clinicians in the WhatsApp groups referenced in planning notes, plus warm introductions from onboarded referrers.
Content Pillars	Referral-win case studies; simple visual guides to the referral workflow; direct answers to clinician questions in-group.
Cadence	Weekly value post in each group; 1:1 follow-up chats with engaged members.
Budget (illustrative)	Organic — relationship-driven, no paid spend.
Sample CTA	"Have a patient who needs a referral today? Message us directly — we'll match them within the hour."

KPI Targets	Referrers actively sending patients; response time to clinician queries.

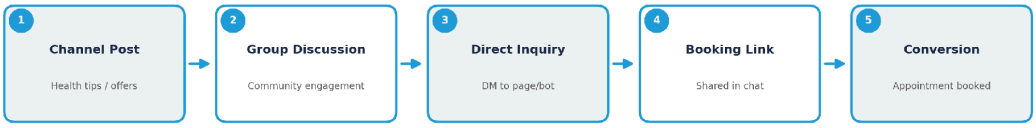


2.4.3 Telegram

Telegram's privacy and lower social friction make it well suited to mental health content on the patient side, and to sustained clinician community-building on the outreach side.

Portfolio A — Patient Acquisition (Community & Nurture)

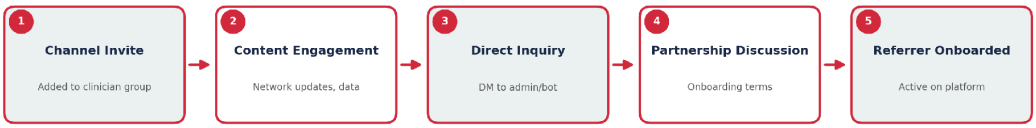
Objective	Build a trusted content community that nurtures prospects who aren't ready to book yet.
Audience	Nigerians seeking mental health information and peer support, plus general health-conscious users.
Content Pillars	Public channel: health education content. Moderated group: peer support and Q&A.
Cadence	3 posts/week in the channel; group moderated daily.
Budget (illustrative)	Organic — no paid spend.
Sample CTA	"Not ready to book? Join the conversation and get answers first."
KPI Targets	Group growth rate; DM inquiry rate; booking-link click-through.



Portfolio B — Clinician & Referrer Outreach

Objective	Maintain an engaged clinician community that refers patients consistently over time.
Audience	Clinicians in the Telegram groups referenced in planning notes.
Content Pillars	Network updates (new hospitals, specialties covered); referral-volume data; quick-reference guides.
Cadence	Weekly network update; admin/bot available for inquiries daily.
Budget (illustrative)	Organic — no paid spend.
Sample CTA	"New specialists just joined the network — see who's available for your next referral."

KPI Targets	Referrer retention; referral volume per active clinician.



2.4.4 LinkedIn

LinkedIn's professional targeting supports two distinct portfolios: reaching the Nigerian diaspora for personal or family bookings (tying into the diaspora market strategy in Section 4), and recruiting clinicians and hospital administrators as referrers.

Portfolio A — Diaspora Patient Acquisition

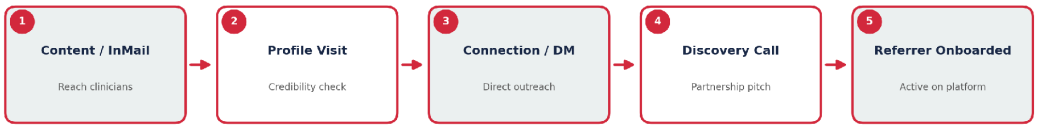
Objective	Convert diaspora professionals into patients booking care for themselves or family in Nigeria.
Audience	Nigerian professionals in the priority diaspora markets (Brazil, India, China, Philippines, Canada) identified in Section 4.
Content Pillars	Sponsored content on booking care for family back home; diaspora testimonials; explainer content on the remote-booking process.
Cadence	1–2 sponsored posts/week; always-on InMail campaign to a targeted diaspora list.
Budget (illustrative)	~~₦~~1,500/week of the ~~₦~~5,000/week test budget — sponsored content and InMail credits.
Sample CTA	"Book trusted care for your family in Nigeria — from wherever you are."
KPI Targets	Cost per lead; application submissions from diaspora markets; cost per booked appointment.



Portfolio B — Clinician & Referrer Outreach

Objective	Recruit clinicians and hospital administrators into the referral network through credibility-first outreach.
Audience	Doctors and hospital admins in relevant specialties and geographies; members of clinician LinkedIn groups referenced in planning notes.
Content Pillars	Thought-leadership posts on Nigeria's referral gap and the provider-network model.

Cadence	2 posts/week; targeted InMail/connection requests sent daily, followed by discovery calls.
Budget (illustrative)	Organic content; InMail credits as the only incremental cost — illustrative ₦500/week.
Sample CTA	"Join a provider network built to fix Nigeria's referral gap. Let's talk."
KPI Targets	Discovery calls booked; referrers onboarded; referral volume per onboarded clinician.



> *Budget splits above are illustrative and total roughly the ₦5,000/week test budget from Section 2.2 across the paid patient-acquisition portfolios (Facebook + LinkedIn diaspora), with WhatsApp and Telegram running organically on both sides. All figures should be reconciled with the bottom-up financial assumptions in the business plan.*

3. Hospital Partnership Strategy

On the provider side, the priority is converting hospital relationships into active use of the Mindset Healthcare platform and a reliable stream of patient referrals.

3.1 Objectives

- Get hospitals to adopt and actively use the platform.
- Get hospitals to send patient referrals and train their staff on the referral workflow.

3.2 Target List

Outreach will prioritize the top 50 hospitals in Nigeria, mapped against the procedures each performs, to ensure the provider network covers the specialties most in demand.

- Compile and prioritize the top 50 hospitals in Nigeria.
- Map each hospital's procedures and specialties to match against expected patient demand.

4. Diaspora Market Strategy

A parallel growth channel targets the Nigerian diaspora — encouraging Nigerians living abroad to use Nigerian hospitals for care, whether for themselves on visits home or for family members in-country.

4.1 Approach

- Identify and reach Nigerians in the diaspora through community and professional networks in each target market.
- Position the top 50 Nigerian hospitals and their procedures as the trusted destination network for diaspora patients.

4.2 Priority Diaspora Markets

Brazil	India	China	Philippines / Canada

These markets were flagged as priority diaspora targets in planning notes. Rationale (size of Nigerian community, remittance patterns, existing ties) should be added once available.

5. Open Items Before Finalization

- Confirm founder/team names and roles for the organizational chart (Section 1.3).
- Validate the campaign budget and CAC assumptions in Section 2.2 against real data.
- Finalize and attach the top 50 hospital list with procedures (Sections 3.2 and 4.2).
- Confirm the rationale and sizing for each priority diaspora market.